

Patient 9**Report A**

65095O125/05/2016 CT head

Clinical history: Fall with left-sided weakness.

Findings: The previous imaging available at time of reporting.

There is high attenuation located within the distal right M1, M2 and several of the peri-insular M3 middle cerebral artery branches in keeping with intravascular thrombus. There is corresponding loss of grey-white matter differentiation of the right anterior temporal lobe and Subjacent insular Cortex with mild surrounding mass effect. No complication of parenchymal haemorrhage.

No intra-axial or extra-axial acute haemorrhage, mass or collection.

No midline shift or hydrocephalus. Comment: Acute right MCA territory infarct with mild surrounding mass effect, no complication of parenchymal haematoma.

Report B

651022226/05/2016 MR Head

Clinical history: Fall, left sided neglect. ?Stroke

Findings:

Movement artefact reduces imaging sensitivity. The patient terminated the study with no intracranial MRA performed.

Comparison has been made with CT head 25/05/2016.

There is mild diffusion restriction located in the right superior and middle temporal gyrus extending to the insular cortex, right supramarginal gyrus and subjacent corona radiata with mild surrounding mass effect, no complication of parenchymal haemorrhage.

No microhaemorrhage, Generalised cerebral sulcal prominence normal for age.

Comment: Within the limitation of this study, there is a right MCA territory infarct with mild surrounding mass effect, no complication parenchymal haemorrhage.